

SYNTHETIC MEDICAL RECORD

Training / Demonstration Data Only — This record is entirely fictional and was created for software testing. It does not describe a real patient, provider, claim, or incident.

Patient: Jordan Miller **DOB:** 04/18/1991 **Case ID:** SYN-PI-2026-001

Reported Incident: Motor vehicle collision on March 12, 2026.

Purpose: Synthetic personal-injury medical record for testing ingestion, OCR, retrieval, structured fact extraction, source grounding, conflict detection, and demand-narrative generation.

Page 1 — Emergency Department Record

Provider: Peachtree Regional Medical Center, Emergency Department

Date of Service: March 12, 2026

History of Present Illness: Jordan Miller presented several hours after a motor vehicle collision. The patient reported being the restrained driver of a vehicle struck from the rear while stopped in traffic. The patient denied loss of consciousness.

Symptoms: The patient reported neck pain rated 7/10, lower back pain rated 6/10, and a headache. The patient described stiffness with turning the head and increased low-back discomfort with prolonged sitting.

Exam: Cervical paraspinal tenderness and lumbar paraspinal tenderness were documented. No focal motor deficit was identified.

Assessment: Acute cervical strain; acute lumbar strain; post-traumatic headache.

Treatment: Acetaminophen was administered. The patient was discharged with instructions for conservative care and outpatient follow-up.

Functional impact: The patient reported that neck and back pain made it difficult to sit comfortably and turn the head while driving.

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Page 2 — Primary Care Follow-Up

Provider: Maya Patel, MD — Midtown Family Medicine

Date of Service: March 16, 2026

Reason for Visit: Follow-up after March 12 motor vehicle collision.

Subjective: Jordan Miller reported persistent neck stiffness, low-back pain, and intermittent headaches since the collision. Pain was worse after sitting at a computer for more than 30 minutes.

Sleep / Daily Activities: The patient reported waking during the night because of back discomfort and needing additional breaks while completing household chores.

Work: The patient works as a software analyst and reported difficulty remaining seated through normal work periods. The patient stated that productivity had decreased because of frequent position changes and short walking breaks.

Assessment: Cervical strain and lumbar strain following motor vehicle collision; headaches.

Plan: Referral to physical therapy. Continue conservative pain management. Return if symptoms worsen.

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Page 3 — Physical Therapy Initial Evaluation

Provider: Atlanta Rehabilitation & Sports Therapy

Date of Service: March 20, 2026

Therapist: Elena Brooks, DPT

Referral: Neck and low-back pain after motor vehicle collision.

Evaluation: Patient reported cervical pain 6/10 and lumbar pain 6/10. Reduced cervical rotation and pain with lumbar flexion were noted.

Treatment: Therapeutic exercise, stretching, manual therapy, postural education, and home exercise program.

Functional limitations: Patient reported difficulty with prolonged sitting, driving longer than approximately 20 minutes, lifting laundry baskets, and sleeping through the night.

Recreation: Patient stated that recreational basketball, normally played twice weekly, had been stopped because running and twisting increased back pain.

Plan: Physical therapy two times weekly for six weeks.

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Page 4 — Physical Therapy Progress Note

Provider: Atlanta Rehabilitation & Sports Therapy

Date of Service: April 3, 2026

Progress: Patient reported neck pain improved to 4/10 but low-back pain remained 5/10 after prolonged sitting.

Treatment: Therapeutic exercise, core stabilization, manual therapy, and home exercise progression.

Functional status: Patient could drive for approximately 30 minutes before needing to change position. Sleep remained interrupted on some nights.

Work: Patient continued to take short standing and walking breaks during the workday.

Recreation: Patient had not returned to basketball.

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Page 5 — Lumbar MRI Report

Provider: Metro Diagnostic Imaging

Date of Study: April 8, 2026

Study: MRI Lumbar Spine Without Contrast

Indication: Persistent low-back pain after motor vehicle collision.

Findings: At L4-L5, there is a small posterior disc protrusion with mild narrowing of the left lateral recess. No acute fracture is identified. Alignment is maintained.

Impression: Small L4-L5 posterior disc protrusion. No acute osseous abnormality.

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Page 6 — Orthopedic Consultation

Provider: Daniel Kim, MD — Georgia Spine & Orthopedics

Date of Service: April 14, 2026

Chief Complaint: Persistent low-back pain and improving neck pain following March 12, 2026 motor vehicle collision.

Record Review: MRI from April 8, 2026 was reviewed and noted to show a small L4-L5 posterior disc protrusion.

Assessment: Lumbar disc protrusion at L4-L5 with mechanical low-back pain; improving cervical strain.

Plan: Continue physical therapy and home exercise program. Consider additional intervention only if conservative treatment fails.

Functional impact: Patient reported continued difficulty with prolonged sitting and had not resumed recreational basketball.

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Page 7 — Physical Therapy Discharge Summary

Provider: Atlanta Rehabilitation & Sports Therapy

Date of Service: May 8, 2026

Course: Patient completed 12 physical therapy visits.

Outcome: Neck symptoms improved substantially. Low-back pain decreased to 2–3/10 with ordinary activity, with occasional discomfort after prolonged sitting.

Function: Patient reported sleeping better, completing household activities with fewer breaks, and tolerating longer periods at the computer.

Recreation: Patient had begun light shooting drills but had not yet returned to full basketball games.

Plan: Discharge to independent home exercise program.

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Page 8 — Billing / Intake Summary with Conflicting Incident Date

Provider: Atlanta Rehabilitation & Sports Therapy — Administrative Intake

Entry Date: March 20, 2026

Patient: Jordan Miller

Incident Date Entered: March 13, 2026

Reason for Care: Neck and low-back pain following motor vehicle collision.

Note: This administrative intake lists the incident date as March 13, 2026. Other clinical records in this synthetic case, including the emergency department and primary-care records, identify the collision date as March 12, 2026. The discrepancy is intentionally included so the software can surface conflicting source facts rather than silently selecting one.

End of synthetic record.